RENEW GROUP OF COMPANIES

RENEW+
HAIR AND SKIN CARE
GET FIXED IN TIME TO SHINE

New Employee Service Training Manual

Clinical services, SOPs and safety standards — Renew Plus Hair & Skin Care

Hydra Facial | Chemical Peels | Microneedling | Skin Boosters & Biostimulators

Induction edition · October 2026 · For internal training use

A Renew Group of Companies initiative.
Manufacturer IFUs, device manuals and the supervising clinician's instructions always take priority over this manual.

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Contents

Welcome & how to use this manual

Part A — Hydra Facial

Indications & client selection · Pre-care · 11-step procedure · Post-care

Part B — Chemical Peels

Product timing & safety · Pre-care · Procedure framework · Rinse-off and leave-on SOPs · Post-care

Part C — Microneedling

Facial area depth guide · Pro tips · Safety notes

Part D — Skin Boosters & Biostimulators

Product classes & brands · Patient selection · Preparation · Techniques · Intervals · Emergencies · Aftercare · Photography · Consent

Part E — Records, checklists & references

Client record template · Pre-treatment checklist · References · Trainee acknowledgement

Welcome to Renew Plus

Welcome to the Renew Plus Hair & Skin Care team. This manual brings together our service SOPs and training modules in one place so you can learn how each treatment works, who it is suitable for, how it is performed safely and how clients are cared for afterwards.

How to use this manual

- Read each Part before you observe or assist with that service. Your trainer will sign you off service by service.
- Learn the **red boxes** first — they are the safety rules that apply every single time.
- Only perform treatments you are trained, authorised and supervised for. Injectables (Part D) and medium/deep peels are clinician-led procedures.

> **The golden rule.** The exact machine manual, product label and current manufacturer Instructions for Use (IFU) always take priority over this general manual. Machine functions, peel strengths, contact times, neutralisation and dilution all differ by product. Never use a generic time or recipe as permission to proceed.

Always

- Screen history, medicines, allergies and recent treatments
- Take consent and baseline photos
- Document products, lots and settings
- Give written aftercare and a contact route

Never

- Improvise neutralisers, mixing or dilution
- Extend contact time because a client "tolerates it"
- Stop a client's prescribed medicine yourself
- Promise a fixed result

PART A

Hydra Facial

14-in-1 / 17-in-1 hydra facial machines — selection, pre-care, procedure and post-care.

1. Indications & client selection

Potential indications: mild congestion, superficial comedones, oily skin, dullness, mild rough texture and temporary hydration / skin-refreshing goals. It is not a substitute for medical acne treatment, pigment diagnosis or treatment of suspicious lesions.

Do not treat / defer

- Active bacterial, fungal or viral infection; cold sore outbreak.
- Open wounds, cuts, sunburn, or an active dermatitis / eczema / rosacea flare.
- Severe sensitivity or compromised barrier; recent aggressive peel, laser or resurfacing before adequate recovery.
- Allergy to a serum or device material; any unexplained rash or lesion.
- Avoid suction over fragile capillaries, bruising tendency, recent filler / threads / surgery, or areas the device manual prohibits.
- Pregnancy, breastfeeding, anticoagulants, immune compromise, poorly controlled diabetes or significant medical conditions — follow device/product guidance and obtain clinician clearance when appropriate.

Special caution: Do not perform suction or exfoliation over active inflammatory acne nodules/cysts, fragile skin or recently treated skin. Do not combine acids, suction and multiple energy modalities simply because the machine has them.

2. Pre-care

- Screen medical history, allergies, skin conditions, current medicines and skincare; ask about recent waxing/threading, sun exposure, injectables, laser, peels and microneedling.
- Ask the client to disclose retinoids, exfoliating acids, benzoyl peroxide, prescription acne medicines and any recent irritation. Follow the device/product protocol for pausing irritating products — do not independently stop prescribed medication.

- For 3–7 days beforehand, a conservative approach is to avoid optional scrubs, home peels and newly introduced strong actives; extend if product/device instructions or the clinician advise.
- Avoid treating sunburned, freshly tanned, irritated or broken skin. No waxing or depilatories immediately before treatment.
- Cleanse and remove makeup. Obtain informed consent and baseline photos. Explain possible temporary redness, tightness, irritation, sensitivity or breakouts.

3. Full procedure workflow (14-in-1 / 17-in-1)

Typical appointment planning time is about **45–75 minutes** including consultation and finishing care. This is a planning estimate, not a device-mandated time. Do not run every attachment just because it is available.

1. **Consultation & skin assessment** (5–10 min)
   Confirm indication, contraindications, allergies and recent procedures. Assess barrier, sensitivity, acne type and pigment risk. Select a conservative plan.
2. **Device & hygiene check** (3–5 min)
   Inspect handpieces, tubing, tips, seals and cables; verify the correct disposable tip/filter and approved solution. Disinfect reusable parts per manual. Never reuse single-use tips.
3. **Gentle cleansing** (3–5 min)
   Remove makeup and sunscreen with a suitable cleanser. Avoid aggressive scrubbing or degreasing unless the protocol requires it.
4. **Optional superficial exfoliation** (3–8 min)
   Device-approved tip/serum and manual settings only. Skip on sensitive, inflamed, recently treated or already-exfoliated skin. Do not combine with a separate chemical peel by default.
5. **Hydrodermabrasion / vacuum cleansing** (8–15 min)
   Smooth, overlapping passes with minimal suction; keep the tip moving. Avoid eyelids, lips, open lesions, inflamed acne and fragile capillaries. Reduce or stop for pain, pinpoint bleeding, marked redness or bruising.
6. **Extraction (optional)** (3–8 min)
   Approved extraction mode only. Do not repeatedly pass over one pore or force extraction. Avoid cystic/nodular and inflamed lesions. Stop if the skin becomes traumatised.
7. **Serum infusion / hydration** (5–10 min)
   Manufacturer-approved, compatible serums only. Check ingredients and allergy history. Never infuse non-sterile products through an unapproved modality.
8. **Optional LED / ultrasound / RF / microcurrent** (only if present and indicated)
   Follow the exact manual, contraindications and settings for that modality — they are not interchangeable. Omit if untrained, contraindicated or skin is irritated.
9. **Mask or soothing finish** (5–10 min)
   Bland, non-irritating moisturising mask. Avoid fragrance or strong actives on sensitised skin.
10. **Moisturiser & sunscreen** (3–5 min)
    Bland moisturiser and broad-spectrum SPF 30–50+, ideally SPF 50+ for pigment-prone clients. Give written aftercare.

11 **Documentation** (2–5 min)
Record device model, attachments, settings, products/lot numbers, suction/modality details, areas treated, skin response, aftercare and follow-up.

4. Post-care

- For 24–48 hours keep the routine simple: gentle cleanser, bland moisturiser and broad-spectrum sunscreen; minimise direct sun.
- Avoid scrubs, exfoliating brushes, home peels, retinoids, AHA/BHA, benzoyl peroxide and other irritating actives for about 2–5 days, or longer if sensitivity persists.
- Avoid picking, squeezing, waxing the treated area, excessive heat/steam and intense exercise for the rest of the day if skin is red or sensitive.
- Mild temporary pinkness or tightness can occur. Clients should contact the clinic for increasing pain, persistent redness, swelling, blistering, rash, discharge or worsening acne. Urgent symptoms or eye exposure need medical assessment.
- Do not promise a fixed result or interval. Repeat only when skin has recovered and the plan supports it.

PART B

Chemical Peels

COS DERMA and MEDI DERMA product timings, universal pre-care, procedure framework, rinse-off and leave-on SOPs, and post-care.

Scope of practice. TCA 15% / 30%, phenol-containing body peels, periocular peels and medium/deep peels require appropriate medical oversight and specific training. This manual does not provide a generic TCA coat count, frosting target or application time.

5. Approximate time & procedure safety by product

Times below are either a verified manufacturer instruction for the named system or a broad planning estimate — never a substitute for the exact bottle/box IFU. Unknown variants are marked **"label required"**.

Product / family	Typical indication	Time guidance	Removal / key safety
COS DERMA Acne Act Gel Peel	Oily/congested skin, comedones, selected acne concerns	One retailer's kit instructions report about 2–5 min; another listing gives 3–10 min. Confirm the exact pack/IFU.	Use only the supplied neutraliser/removal steps. Never extend time because the client tolerates it.
COS DERMA Salicylic / Mandelic / Glycolic	Acne/congestion, texture or uneven tone depending on formulation	Label required — concentration and pH vary.	Do not assume all are rinse-off or share the same neutraliser.

Product / family	Typical indication	Time guidance	Removal / key safety
COS DERMA Part Gel / Melas / Whitening Peel	Depends on exact formulation and indication	Label required.	Confirm full name, strength, pH, approved site and removal instructions.
COS DERMA Under Eye Peel	Only specific periocular concerns approved by the manufacturer	Label required; do not estimate.	High caution: avoid eye contact and lid margin; trained, authorised clinician only.
COS DERMA Retinol Peel	Selected texture / photoageing / pigment concerns	Label required; do not assume leave-on duration.	Confirm leave-on vs wash-off, sealing product and exact wash time.
COS DERMA TCA face/body (15% / 30% if on label)	Selected indications under qualified medical assessment	No generic duration can be safely assigned.	Restricted, clinician-led. No generic coat count, frosting endpoint or neutralisation method.
MEDI DERMA ArgiPeel	Selected superficial texture/brightness concerns	Use current product IFU; confirm exact variant.	Follow the manufacturer rinse-off protocol and product-specific aftercare.
MEDI DERMA Ferulac Peel Booster System	Photoageing, sunspots, selected hyperpigmentation	Official system: coats dry 3–4 min between layers; Retises Nanopeel 1% gel left on about 8 h.	Multi-step system — use exact kit components and IFU, then wash after the specified time.
MEDI DERMA Retises CT Yellow Peel	Selected ageing/pigmentation concerns	Official instructions: 6–8 h for the retinol cream step, then wash.	Retinoid-containing; pregnancy/possible pregnancy requires avoidance and clinician review.
MEDI DERMA Nomelan / "No Melan" body peel	Cannot confirm without exact product name and variant	Label required.	Don't confuse post-care cream NOMELAN C with a professional peel, or body products with facial systems.

Important distinction: "Time duration" can mean total appointment time, acid contact time, time between coats, or leave-on time at home. These are not the same. Never convert a published time for one variant into a protocol for a different peel.

6. Universal pre-care

- Assess skin type, pigment risk, barrier health, prior PIH/scarring, allergies, pregnancy possibility, medications and all recent treatments.
- Defer for active infection/cold sore, open skin, sunburn, active dermatitis, severe irritation, unexplained lesions or inadequate healing from a prior procedure.
- Review prescribed acne medicines, isotretinoin history, retinoids, acids, benzoyl peroxide and photosensitising medicines with the qualified clinician; do not stop prescriptions yourself.
- Avoid tanning and protect from sun. Follow the product's preconditioning directions; avoid new active products right before treatment.

- Explain expected flaking/redness and risks including burns, infection, PIH, hypopigmentation and scarring. Obtain informed consent and baseline photographs.
- Verify product name/variant, concentration, expiry, batch, storage, IFU, approved site, neutraliser/removal method and emergency plan **before opening**.

7. General procedure framework

- Cleanse with the product-directed cleanser; dry completely if the IFU requires.
- Protect sensitive areas only as the manufacturer directs. Prepare timer, approved applicator, neutraliser/rinse, gloves and emergency irrigation supplies.
- Apply only the authorised amount, sequence, layers and area. Keep product away from eyes, mucosa and unintended sites. Do not layer multiple peel brands unless the official protocol directs it.
- Observe continuously. Stop for severe/escalating burning or pain, unexpected whitening/frosting, blistering, marked swelling or other signs of injury, and follow the product's emergency instructions.
- Neutralise or remove only if and how the product instructions specify — some are leave-on/sealed systems, others rinse-off or neutralised. Do not guess.
- Apply the approved finishing product and sunscreen. Give written wash-off time, post-care, warning signs and contact details.
- Document exact product and lot, site, coats (only where specified), timing, removal/neutralisation, skin response, aftercare and review.

8. SOP for neutralised / rinse-off peels

Use this pathway only when the exact product IFU states the peel is neutralised or rinsed off. The IFU overrides this general sequence.

- Verify product identity, matching neutraliser/rinse instructions, permitted area, application method, exposure endpoint, maximum exposure and emergency response before opening.
- Prepare skin exactly as specified. Do not add degreasing, scrubbing or pre-treatment acids unless the IFU requires them.
- Use only manufacturer-specified barrier protection. Avoid eyes, nostrils, lips, mucosa and compromised skin unless specifically approved for those areas.
- Observe continuously. Ask about pain/burning and inspect frequently — sensation alone does not reliably indicate peel depth.
- At the IFU-defined endpoint — or sooner if an adverse response occurs — use the specified neutraliser or rinse exactly as directed. Never use household chemicals or an improvised neutraliser.
- Reassess and document endpoint, response, coverage, timing, neutralisation/rinsing and any deviation.
- Apply only approved post-peel products and provide written aftercare plus a contact route for delayed reactions.

Emergency: If severe pain or an unexpected reaction occurs, stop and follow the product's emergency decontamination instructions. For eye exposure, start immediate continuous irrigation and arrange urgent emergency assessment — do not delay irrigation.

9. SOP for retinol / retinoid leave-on peels

- Confirm the exact product is labelled as a leave-on retinol/retinoid peel and whether it is professional-use-only.
- Review current retinoids, recent exfoliants, barrier status, pigment history, sun exposure and product-specific contraindications. Defer irritated or compromised skin.
- Record exact product/formulation, lot/expiry, intended area, application amount, prescribed leave-on period and client removal instructions.
- Apply only as directed by the IFU and within your authorised scope. Do not layer with another peel or device exfoliation unless explicitly supported.
- Do not neutralise or wash off based on a generic rule. Follow the product-specific removal schedule, including written home-removal instructions.
- Explain that dryness, redness, sensitivity and peeling may be delayed. Avoid retinoids, acids, scrubs and other irritants during the IFU-specified recovery period.
- Arrange follow-up and give a clear contact route for intense burning, swelling, blistering, eye symptoms or worsening pain.

10. Chemical peel post-care

- Gentle cleanser, bland moisturiser and broad-spectrum SPF 50+. Avoid deliberate sun exposure; reapply sunscreen as directed.
- Do not pick or pull peeling skin. Avoid scrubs, waxing, depilatories, exfoliating acids, retinoids, benzoyl peroxide and strong brightening products until fully recovered and the clinician permits restarting.
- Follow the manufacturer's wash-off instructions for leave-on systems. Never leave a product on longer to increase results.
- Seek prompt clinical review for increasing pain, severe/persistent redness, blistering, weeping, pus, fever, cold sores, eye symptoms or worsening pigment change. Eye exposure requires immediate copious irrigation and urgent medical/ophthalmic assessment.

PART C

Microneedling

Facial area depth reference and practical tips — there is no one-size-fits-all protocol.

Understanding the zones

Delicate areas — thinner skin needs shallower depths for safety and comfort (brows 0.25 mm, nose 0.25 mm, under-eyes 0.25–0.5 mm, upper lip 0.25–0.5 mm, forehead 0.25–0.75 mm).

Moderate areas — target texture, pigment and early signs of ageing (cheeks 0.75–1.25 mm).

Deeper areas — firmer, more resilient skin (jawline 0.75–1.5 mm, chin 0.5–1.0 mm, neck 0.5–1.5 mm). Acne scars and stretch marks are customised.

Pro tips

- Assess skin thickness and condition first.



Microneedling area guide — reference needle-depth ranges by facial zone.

- Adjust depth to the treatment goal.
- Use sterile, single-use cartridges.
- Work in sections with even pressure.
- Follow proper aftercare and recovery time.

Safety: Depths shown are reference ranges only. The final depth, device setting and number of passes are set by the trained practitioner according to the device IFU and the client's skin. Apply the same screening rules as other services — defer active infection, cold sores, inflamed acne, open or sunburned skin, and recent peels or resurfacing. Do not combine microneedling with peels or topical actives unless the protocol specifically supports it.

PART D

Skin Boosters & Biostimulators

Cross-linked HA • Non-cross-linked HA • CaHA • PLLA • PDRN • PN — clinical education reference for qualified medical professionals.

Important: This is not a standalone injection course. It intentionally does not provide universal needle-entry maps, exact injection depths, per-point volumes or mixing recipes — these vary by product, anatomy, indication and regulatory labelling, and a generic recipe can cause serious harm. Injectables are performed only by qualified, trained medical practitioners.

11. Learning objectives

By the end of this module you should be able to: distinguish hydration-focused skin boosters from volumising fillers and collagen biostimulators; screen and consent patients; identify when a product must be used as supplied versus reconstituted; plan review intervals; and recognise complications requiring urgent escalation.

12. Product classification

Product class	Typical purpose	Preparation principle	Key caution
Non-cross-linked HA	Hydration, superficial skin quality and fine lines	Often supplied ready-to-use; only modify if the IFU explicitly permits	Shorter persistence; papules, swelling, infection and vascular injury remain possible
Cross-linked HA	More persistent hydration/structure; some products add volume	Usually a prefilled gel; do not dilute unless the IFU allows	Product-specific rheology and plane; vascular occlusion risk
CaHA	Structural support and collagen stimulation	Use as supplied unless the exact IFU supports a specified dilution	Not a routine lip/tear-trough product; nodules and vascular injury are concerns
PLLA	Gradual collagen stimulation and volume restoration	Product-specific reconstitution; exact diluent/volume/time per IFU	Delayed nodules/granulomas; results are gradual
PDRN / PN	Regenerative/skin-quality claims; evidence and approvals vary	Only a sterile injectable intended for that route; no improvised mixing	Protocols not standardised; check legality, allergy history and evidence

Brand examples by category

Brand examples are educational, not endorsements. Availability, registration, composition, indications and IFU may differ in India and by product version — verify the actual pack, current local IFU and regulatory status before use.

Category	Brand / product examples	General positioning	Preparation caution
Non-cross-linked HA / skin-quality injectables	Teosyal Puresense Redensity 1; selected non-cross-linked HA biorevitalisers	Hydration and skin-quality improvement; exact indication is product-specific	Often ready to use. Do not add saline or another HA unless the IFU permits.
Cross-linked HA skin boosters	Restylane Skinboosters Vital / Vital Light; Juvéderm Volite (market-dependent naming)	Selected products improve hydration/quality; some are stabilised or cross-linked	Generally use as supplied. Never infer a dilution recipe from a different HA product.
Hybrid / cooperative-complex HA	Profhilo (IBSA)	Stabilised hybrid HA complex; not interchangeable with conventional cross-linked filler or non-cross-linked HA	Use the manufacturer's specific technique and instructions; do not improvise mixing.

Category	Brand / product examples	General positioning	Preparation caution
CaHA biostimulator	Radiesse / Radiesse (+) (Merz)	CaHA microspheres in gel carrier; selected contour/volume and collagen-stimulation uses	Some protocols allow dilution for defined indications. Do not generalise ratios to other areas/products.
PLLA biostimulator	Sculptra; Lanluma (market-dependent)	Gradual collagen stimulation over weeks to months	Reconstitution differs by product. Follow exact diluent, volume, wait time, storage and use window.
PN / PDRN injectables	Rejuran (PN range); Plinest / Plinest Fast (PN range); selected PDRN products	Regenerative/skin-quality claims; formulation and evidence vary between PN and PDRN	Check authorisation for injection and site. Do not mix with HA or saline unless the IFU permits.

Key formulation distinctions

- HA is not one uniform material — cross-linking, concentration, gel structure, cohesivity and lidocaine content change handling and intended use.
- Profhilo is a stabilised hybrid HA complex and should not be labelled as the same as conventional cross-linked fillers.
- CaHA and PLLA are collagen biostimulators, not HA gels; **hyaluronidase does not dissolve them**.
- PN and PDRN are related but not identical. A topical cosmetic ampoule is not automatically suitable for injection.

13. Patient selection, indications & contraindications

Potential indications (only when consistent with the product IFU)

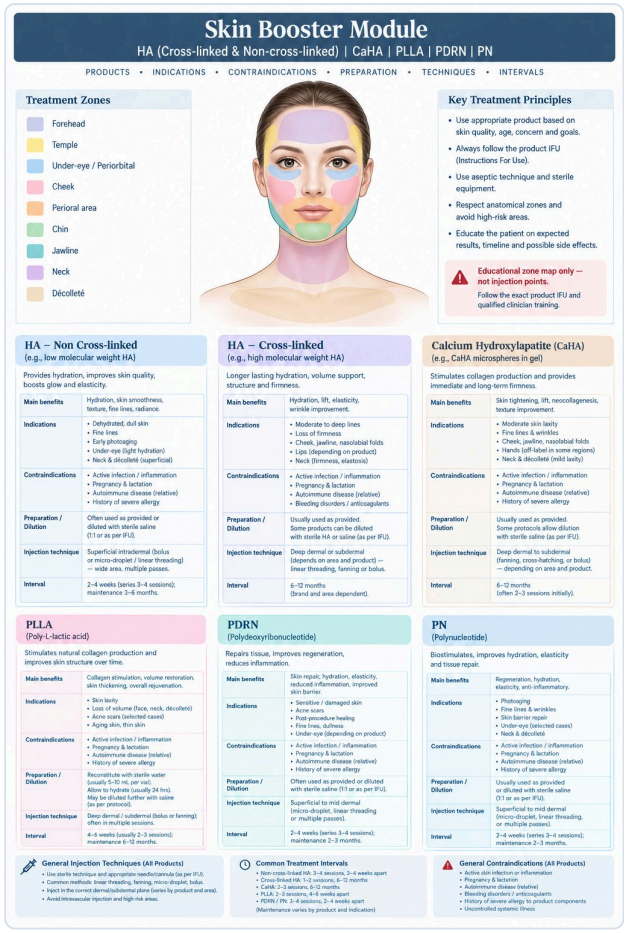
- Dehydrated or dull-looking skin and selected fine lines (HA-based products).
- Selected facial folds or volume/contour loss (specific cross-linked HA or CaHA products).
- Skin laxity and texture concerns where a collagen biostimulator is indicated (CaHA/PLLA).
- Selected skin-quality indications for PN/PDRN where locally authorised and evidence-supported.

A product's marketing name alone does not establish its approved indication.

Defer treatment and obtain medical assessment

- Active skin, dental or systemic infection; active herpes at or near the area; unexplained rash or inflammation.
- Pregnancy or breastfeeding — elective cosmetic injectables are generally deferred.
- Known severe allergy to ingredients; for fish-derived PN/PDRN, clarify source and allergy history.
- Bleeding disorder, anticoagulant/antiplatelet use, immune suppression, uncontrolled systemic illness, prior severe filler reaction, autoimmune or granulomatous disease — individual medical risk assessment required.
- Unrealistic expectations, inability to consent or to attend follow-up.

Always review prior fillers/biostimulators, previous complications, medications, allergies, recent procedures, scarring tendency and the patient's goals. Contraindications differ by product; the IFU takes precedence.



Skin Booster Module overview. Educational zone map only — not injection points. Dilution notes in this graphic are illustrative; always follow the exact product IFU (see Section 14).

14. Dilution & reconstitution decision guide

There is no universal "saline method" or "HA method" for all product groups. Dilution changes concentration, spread, handling and risk; mixing products may be off-label and can create incompatibility or contamination risks.

Product	General preparation rule	What not to do
Non-cross-linked HA	Use the supplied formulation unless the manufacturer explicitly authorises dilution/mixing.	Do not assume saline or another HA can be added safely.
Cross-linked HA	Most are used as supplied. Follow the exact label for any permitted modification.	Do not dilute a filler to make it behave like a skin booster without product-specific guidance.
CaHA	Some labels allow defined dilution for particular indications — follow the exact approved ratio and area.	Do not generalise one product's ratio to other brands, sites or indications.
PLLA	Reconstitute with the exact sterile diluent, volume, preparation time and storage rules in that IFU.	Do not substitute HA for the specified diluent unless the IFU permits.

Product	General preparation rule	What not to do
PDRN / PN	Use as supplied unless its own IFU permits a specific preparation.	Do not mix with HA, saline, lidocaine or other injectables based on social-media protocols.

What the brand changes — examples

- **HA:** Restylane Skinboosters, Juvéderm Volite and Teosyal Redensity 1 are generally supplied in a prepared injectable format. Do not assume they should be diluted or mixed.
- **CaHA:** Official Radiesse materials describe a specific 1:2 dilution with 0.9% sterile saline for a defined décolleté indication in the US — not a universal face/neck recipe and not automatically an Indian-approved indication.
- **PLLA:** Official Sculptra labelling specifies reconstitution with sterile water for injection. Sculptra and Lanluma each have their own instructions — never transfer volume, hydration time or storage rules between brands.
- **PN/PDRN:** Rejuran and Plinest lines include different formulations — do not assume the same area, technique, dilution permission or interval across a range.

No universal HA-mixing method. Adding HA to another injectable may be off-label and may alter viscosity, spread, stability or safety. Never mix products in the same syringe based on informal protocols or social media.

Safe preparation checklist

1. Verify brand, batch, expiry, authenticity, storage and local regulatory status.
2. Confirm it is a sterile injectable product — not a topical ampoule or cosmetic serum.
3. Read the current IFU for diluent, compatibility, reconstitution, hold time, storage and single-use requirements.
4. Use aseptic technique and sterile single-use supplies; do not reuse or save leftover product unless the IFU permits.
5. Document product name, lot, expiry, volume used, treatment area and consent.

15. Injection technique concepts

Technique depends on product viscosity, particle characteristics, intended tissue plane, area and the clinician's anatomical training. Common terms include superficial intradermal microdroplets, linear threading, fanning, serial puncture and subdermal placement — these are not instructions to use the same technique for every product.

Before treatment

- Review facial vascular anatomy and danger zones in formal hands-on training.
- Mark only after anatomical assessment; never use generic internet maps as a substitute for individual anatomy.
- Ensure emergency supplies, an escalation pathway and medical support are available.
- Use slow, low-pressure injection where the product's training/IFU supports it; stop immediately for unusual pain, blanching, livedo, visual or neurological symptoms.

A cannula is not risk-free, aspiration does not guarantee against intravascular placement, and superficial placement is not automatically safe. High-risk areas — especially nose, glabella, forehead and periocular region — require advanced medical training and may be outside product approvals.

16. Treatment intervals & review planning

Intervals are product-, indication- and patient-dependent. These are broad planning examples only — follow the IFU and the treating clinician's assessment.

Class	Commonly used planning pattern (verify IFU)	Review focus
Non-cross-linked HA	Often a short initial series spaced several weeks apart; maintenance may be needed every few months. Some protocols use a single treatment.	Hydration, texture, papules, swelling and satisfaction
Cross-linked HA	Often one session, review around 2–4 weeks; repeat only when clinically indicated and consistent with IFU.	Symmetry, lumps, delayed inflammation, vascular warning signs
CaHA	Often one session or a limited series; review around 4–8 weeks, with longer-term assessment over months.	Laxity, contour, nodules and delayed inflammatory changes
PLLA	Usually a staged series several weeks apart; collagen effect develops over weeks to months.	Gradual response, nodules, asymmetry, delayed complications
PDRN / PN	Published protocols vary; some use a short series at roughly 2–4-week intervals — no universal standard across brands.	Skin-quality outcomes, reactions, evidence and product safety

17. Adverse events & emergency response

Expected short-term effects include tenderness, redness, swelling, bruising and small injection-site bumps — explain these before consent. Persistent or worsening symptoms need clinical review.

Urgent red flags

- Sudden or disproportionate pain; blanching, mottled/livedoid or dusky skin; cool skin or delayed capillary refill.
- Any visual disturbance, eye pain, severe headache, weakness, facial droop, speech difficulty or other neurological symptom.
- Rapidly worsening swelling, spreading redness, fever, pus, skin breakdown or delayed painful nodules.

If vascular occlusion is suspected: stop injecting immediately and activate the clinic's emergency protocol. Visual or neurological symptoms require **immediate emergency transfer** — do not wait to see if they resolve. For HA complications, trained clinicians may use hyaluronidase per current emergency guidance; it does not dissolve CaHA or PLLA. Never improvise treatment or delay referral.

18. Aftercare & documentation

- Give written, product-specific aftercare and a 24-hour contact route.

- Avoid manipulating or massaging the area unless the product protocol explicitly instructs it.
- Patients should contact the clinic for increasing pain, unusual colour change, worsening swelling, fever, discharge, or visual/neurological symptoms (emergencies).
- Record baseline and follow-up photos, consent, product/lot/expiry, amount used, exact areas, immediate response, advice given and planned review.
- Schedule product-appropriate follow-up and document any adverse event and referral.

19. Clinical photo protocol

Standard photo set

1. Frontal face at rest.
2. Right and left oblique views (~45°).
3. Right and left profile views.
4. Close-up of the intended area before treatment.
5. Immediate post-treatment images if useful (swelling/bruising).
6. Follow-up images at the same angles, lighting, distance and expression.

Photo marking rules

- Use the patient's own anatomy; mark only in person after assessment.
- Label the product and indication, not a generic "booster point".
- Keep identifiers out of teaching images unless documented permission exists.
- Never promise an exact result from a pre-treatment image.

Facial-zone infographics are for documentation and discussion only — never for choosing needle entry points or depths.

20. Consent checklist

Confirm and document: indication and alternatives; expected benefit and limitations; off-label status if applicable; product identity; common reactions; nodules / infection / allergy; vascular occlusion, skin necrosis, visual injury and stroke risk where relevant; possible need for urgent treatment; expected onset/duration; costs and follow-up; photo consent; and the patient's understanding and questions.

PART E

Records, Checklists & References

Templates to use on the floor and sources for product-specific information.

21. Client record template

Field	Entry
Client / date / operator	
Indication / skin type / baseline photos	
Contraindication screening / medicines / allergies	

Field	Entry
Device model and modalities OR exact peel / product name, variant, strength	
Batch / expiry / IFU checked	
Settings / application details / timings per IFU	
Removal / neutralisation / leave-on wash time	
Immediate response / adverse event	
Pre- and post-care explained / consent	☐ Yes ☐ No
Follow-up plan	

22. Pre-treatment checklist (injectables)

☐ Qualified injector and product-specific training verified
☐ Current IFU and local authorisation reviewed
☐ Patient history, medications, allergies and previous injectables reviewed
☐ No active infection/inflammation; pregnancy/breastfeeding status considered
☐ Baseline photographs and consent completed
☐ Product, batch, expiry, authenticity and storage checked
☐ Sterile supplies and emergency plan available
☐ Follow-up and aftercare explained

23. References

- COS DERMA Acne Act Peel — official product page and kit listings (confirm the instructions supplied with your exact kit).
- MEDI DERMA Ferulac Peel Booster System and Retises CT Yellow Peel — manufacturer pages.
- American Academy of Dermatology — preparing for a chemical peel, recovery and FAQs.
- U.S. FDA — Dermal Fillers (Soft Tissue Fillers): fda.gov/medical-devices/aesthetic-cosmetic-devices/dermal-fillers-soft-tissue-fillers
- U.S. FDA — Sculptra labelling/approval summary: fda.gov/medical-devices/recently-approved-devices/sculptra-p030050s039
- Radiesse (Merz) — provider resources: radiesse.com/professionals/practice-resources
- Galderma — Restylane Skinboosters; Allergan Aesthetics India — Juvéderm; Teoxane — HCP product information.
- Association of Cutaneous Surgeons of India Task Force — consensus position statement on emerging treatments in dermatology (2026).
- Systematic review: The Effectiveness of Polynucleotides in Esthetic Medicine — pmc.ncbi.nlm.nih.gov/articles/PMC11845969

Clinical governance: This manual is a general training overview, not a substitute for manufacturer IFUs, hands-on anatomical training, medical judgement, local regulations or a formal complication-management course. Verify current local approvals and exact product labelling before every treatment.

24. Trainee acknowledgement

I confirm that I have read and understood this training manual, and that I will perform only those services for which I have been trained, assessed and authorised.

Employee name	_____
Role / department	_____
Employee signature & date	_____
Trainer name & signature	_____
Services signed off	☐ Hydra Facial ☐ Chemical Peels ☐ Microneedling ☐ Skin Boosters (clinicians)

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